PUBLIC COMMENT — RFI RESPONSE

Submitted to the Centers for Medicare & Medicaid Services via regulations.gov

Re: Response to Request for Information III.C (Improving Implementation of Payer Application Programming Interface Technology)

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Submitted by:

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This RFI response should be read together with EVRESA's foundational public comment on CMS-0062-P. This filing addresses Request for Information III.C specifically and provides additional architectural input on payer Application Programming Interface implementation, with emphasis on the proposition that APIs alone do not prove whether the right authority reviewed, approved, denied, escalated, or modified a prior authorization decision.

I. Context for This Response Request for Information III.C in CMS-0062-P invites public input on improving the implementation of payer Application Programming Interface technology, including challenges with existing API implementations, completeness and reliability of API responses, and methods to ensure that standardized APIs deliver the interoperability benefits CMS-0057-F and CMS-0062-P intend. EVRESA's response addresses an architectural consideration that has emerged from observed CMS-0057-F implementation experience:

APIs alone do not prove whether the right authority reviewed, approved, denied, escalated, or modified a prior authorization decision. They prove only that a transaction occurred. The architectural question CMS-0062-P implementation raises is whether the standardized API environment can be paired with governance proof infrastructure that demonstrates the authority basis for the decisions transiting those APIs.

Requested CMS Action

EVRESA respectfully requests that CMS recognize in future sub-regulatory payer API or implementation guidance that standardized prior authorization APIs may benefit from governance-layer controls operating above the API layer, including authority attestation, decision provenance, patient identity governance references, reliability compensation, and authority completeness verification. EVRESA does not request that CMS mandate any specific API technology, vendor, or implementation architecture.

II. The API-Authority Distinction Standardized APIs solve a specific problem: they enable consistent data exchange across diverse systems through documented interface specifications, predictable response formats, and reliable transaction protocols. Standardized APIs do not solve a different problem: they do not establish the authority basis for the data being exchanged. A FHIR Da Vinci Prior Authorization Support transaction can transmit a denial reason in standardized format without proving that the denial reflects authorized human medical judgment. A NCPDP SCRIPT message can transmit a prior authorization status without documenting which reviewer authorized the status determination.

This distinction matters because CMS-0062-P implementation creates an environment in which API completeness and reliability become regulatory expectations, while authority attestation remains an implementation consideration largely left to individual entity discretion. Stakeholder reporting from

CMS- 0057-F implementation through 2025 has surfaced concerns about both dimensions: API implementations that are technically compliant but produce incomplete or inconsistent responses, and decision documentation that meets transaction standards but does not provide audit-defensible authority chains.

II.A. API Completeness Versus Authority Completeness API completeness considerations focus on whether the standardized response includes all data fields the standard specifies. Authority completeness considerations focus on whether the data fields, even when complete, document the authority basis for the underlying decision. A response can be API-complete and authority-incomplete simultaneously: the denial reason field is populated, but the field does not indicate whether the denial reflects human medical judgment or automated inference operating without human review.

Governance proof infrastructure operating above the standardized API environment can address authority completeness without requiring changes to the standardized API specifications. The governance layer attests to authority, evidence, and human review for the decisions transiting the API. The API itself remains unchanged. Recipients of API responses can verify authority completeness by referencing the governance attestation rather than attempting to extract authority information from API fields that were not designed to carry it.

III. Patient Identity and Decision Provenance Payer API implementation also raises considerations about patient identity and decision provenance that the foundational comment introduces in Section IV.A and that this RFI response can address with additional depth. In standardized API environments, decisions reference patient identity through identifiers that may operate across multiple systems with different identity governance approaches. The connection between the patient identity record, the authority of the reviewer, the evidence reviewed, and the decision outcome is a governance question that API standardization does not fully resolve.

Governance-layer patient identity references. A governance-layer approach to API implementation can allow prior authorization events to be linked to a governed patient identity reference and corresponding decision receipt without requiring changes to the underlying patient identity systems each entity maintains. The composition pattern is similar to the FHIR/NCPDP composition described in the foundational comment: existing patient identity systems remain the system of record for patient identity, while the governance layer adds an attestation reference that connects identity, authority, evidence, and decision into an auditable evidence package.

This pattern addresses an important API implementation challenge: ensuring that decisions transiting the standardized environment are properly tied to the patient whose care they affect, with sufficient identity verification for downstream entities to act appropriately on the decision. CMS need not prescribe any specific identity implementation in CMS-0062-P, but the agency may consider, in subsequent sub- regulatory guidance, the architectural pattern of governance-layer patient identity references as one approach to addressing this implementation challenge.

IV. API Reliability and the Network-Level Governance Service Stakeholder reporting from CMS-0057-F implementation through 2025 has surfaced significant variation in API reliability across impacted payers. Some payers have invested heavily in robust API implementations; others have produced API implementations that are technically compliant but operationally fragile. The variation creates implementation challenges for entities that depend on consistent API behavior, particularly independent pharmacies, smaller provider organizations, and entities operating without proprietary internal compliance infrastructure.

Network-level governance services can mitigate API reliability variation. A governance proof layer operating as a network-level service can provide consistent governance attestation across decisions transiting payer APIs of varying reliability. Where a payer API response is incomplete or inconsistent, the governance layer can detect the inconsistency, route the decision to human review, and produce an attested record of the resolution. This pattern preserves the standardized API as the transaction layer while providing reliability compensation at the governance layer.

This is the architectural answer to the asymmetry the foundational comment identifies in Section IV.B. Entities with internal compliance infrastructure can manage API reliability variation through internal systems. Entities without internal compliance infrastructure benefit from network-level governance services that provide consistent attestation regardless of which payer API they interact with on a given transaction.

V. Suggested CMS Guidance Language In the spirit of providing actionable input to the rulemaking process, EVRESA respectfully offers the following suggested language that CMS may consider for inclusion in subsequent sub-regulatory payer API or implementation guidance accompanying CMS-0062-P. This language is offered for CMS consideration without expectation of adoption and may be adapted, modified, or used in part as CMS deems appropriate.

> *"CMS recognizes that standardized payer Application Programming Interfaces enable consistent transaction-layer data exchange but do not, by themselves, establish the authority basis for the decisions transiting those interfaces. CMS encourages impacted payers, providers, pharmacies, and infrastructure providers to consider governance-layer controls that complement standardized APIs, including documentation of authority attestation, evidence linkage, patient identity governance references, and authority completeness for decisions transmitted through prior authorization APIs. CMS does not prescribe any specific API technology, vendor, or implementation architecture."*

VI. Conclusion and Recommendation EVRESA's response to RFI III.C advances a single architectural argument: APIs alone do not prove authority, and the standardized API environment CMS-0062-P creates is most effective when paired with governance proof infrastructure that addresses authority completeness, patient identity attestation, and reliability compensation at a

layer above the transaction environment.

Recommendation. EVRESA respectfully recommends that CMS recognize, in any sub-regulatory guidance addressing payer API implementation under CMS-0062-P, that architectural patterns operating above the API layer can address authority attestation, patient identity governance, and reliability compensation considerations that API standardization alone does not fully resolve. Recognition of this architectural pattern as a regulatory consideration provides regulatory clarity to entities seeking to operate effectively in the standardized API environment the rule creates without requiring CMS to mandate any specific API implementation technology or vendor solution.

Respectfully submitted, John Gray Rodriguez

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Disclosures. This RFI response is submitted in good faith on the technical and architectural merits of the considerations raised in the proposed rule. Disclosures are consistent with EVRESA's foundational comment on CMS- 0062-P, including absence of federal grants or contracts, absence of pending federal agency engagement, absence of commercial relationships that materially bear on the recommendations advanced herein, absence of compensation arrangements for this comment, and Section 508 accessibility consistent with HHS guidelines for materials posted on regulations.gov.